TakeSure — Pitch script that walks through the abstract

Hack@BVP 8.0 · Round 1 (Discord, 18 Oct 2026) · Team Bug Hunters · Use with **TakeSure_Abstract_HackBVP8.pdf** on screen, then the live site

How to use this. Share the abstract PDF on screen. Each block below matches one part of the page, top to bottom. Speak the words in the "Say" lines; the grey line tells you what to point at. Full version is about 5 minutes (final judging). For the 3-minute mentoring slot, skip the blocks marked **[3-min: skip]**. The live demo replaces the flowchart block when the site is running.

Timing

Time	Part of the PDF	What you do
0:00–0:20	Header band	Name, tagline, team, domain
0:20–1:00	Story strip (4 panels)	Tell the mother's story, one panel each
1:00–1:40	1 · The problem + 4 numbers	Two systems, no check; the statistics
1:40–2:20	2 · Proposed solution + the rule	Five verbs: Scan, Match, Decide, Explain, Report
2:20–3:20	3 · Workflow flowchart	Walk left to right; or switch to the live demo here
3:20–4:00	Comparison table + knowledge-base table	Why nobody else does this; what a KB row looks like
4:00–4:40	4 · Expected impact	Patients, doctors, Ministry, novelty
4:40–5:00	Close	One line and the ask

The script

0:00 · HEADER BAND

Point at the title, then the pill × leaf → shield icon, then the domain strip.

"Good morning. We are Team Bug Hunters — Dhriti, Shiwani and Khushi. Our project is TakeSure: confidence before taking something. It is a safety checker for the one combination India takes every day and nobody checks: an Ayurvedic product together with an allopathic medicine. We are in the Health and Well-Being domain, SDG 3."

0:20 · ONE PATIENT'S STORY (four panels)

Point at panel 1, 2, 3, 4 in turn. Slow down on panel 2.

"Let me start with one patient. **Panel one:** a mother takes Thyronorm for her thyroid, and starts ashwagandha for stress because a relative suggested it. **Panel two:** her endocrinologist never asks about herbs, she never mentions it, and the ashwagandha label says nothing about thyroid tablets. Nobody asks, nobody tells. **Panel three:** with TakeSure she photographs the prescription and the ashwagandha box. The app reads the names, shows them back to her, and she confirms. **Panel four:** she sees AMBER — caution — with a published source, advice on timing, and three questions for her next appointment. One tap sends a PDF to her doctor. That is the whole product in four pictures."

If you have a real Thyronorm strip and an ashwagandha bottle, hold them up on panel one. It works on camera.

1:00 · SECTION 1 — THE PROBLEM

Point at the four bullets, then the four numbers.

"Why does this matter beyond one family? India runs two medical systems side by side and there is zero cross-checking between them. Ashwagandha can raise thyroid hormone, giloy can work against transplant drugs like tacrolimus, guggul can increase bleeding with warfarin. The numbers: 37 percent of hospital patients use both systems together. 65 percent of herbal users do not know the dose, side effects or interactions. Only 19 percent ever tell their doctor. And the national Ayush pharmacovigilance programme has received about 1,473 adverse-event reports since 2018 — for a country of 1.4 billion. The tools that exist do not cover it: Medscape, Drugs.com and 1mg check allopathic against allopathic only, and Western herb databases are English-only, paid, and have never heard of Himalaya or Dabur."

1:40 · SECTION 2 — PROPOSED SOLUTION

Point at the five bold words, then the yellow rule box.

"TakeSure is a phone-first safety layer between the two systems, and it does five things. **Scan both**: photograph the prescription and the product label, or just type the names; every name is confirmed before anything is checked. **Match Indian names**: Thyronorm becomes levothyroxine, Chyawanprash becomes amla and ashwagandha, with Sanskrit, Hindi and Latin synonyms. **Decide from data**: a rule engine checks every herb–drug pair against a curated, cited knowledge base and gives GREEN, AMBER, RED, or 'not in database'. **Explain simply**: AI rewrites only the matched evidence into plain Hindi or English with three questions for the doctor. **Report in one tap**: a PDF for the doctor and a pre-filled suspected-reaction report in the Ministry of Ayush format. And the one rule that makes all of this trustworthy is in the yellow box: **the verdict comes from published data; AI only explains it.**"

2:20 · SECTION 3 — THE WORKFLOW FLOWCHART

Trace the flowchart left to right with the cursor. Pause on the dark "Rule engine" box.

"Here is how it works inside. Two photos come in on the left. Step one, OCR reads them — we use Tesseract with English and Hindi, and the user confirms every name. Step two, matching: a bridge dictionary plus fuzzy matching turns brand names into generics and products into herbs. Step three, the knowledge base: every row has a source with a DOI or PubMed ID, a severity, and an evidence grade A, B or C. Then the dark box — the rule engine — decides. Worst row wins. An unknown pair is never shown as green. Weak grade-C evidence can never produce more than AMBER. Only after the verdict is fixed does the AI step in, in step four, to explain it in simple language — it cannot add, remove or re-grade an interaction. Switch the AI off and the answer is identical. On the right are the two outputs: the doctor PDF and the ADR report."

When the site is running, say "let me show you" here and do the live flow: Check safety → Type the names → + Thyronorm, + Ashwagandha → Match → Confirm → AMBER result → click हिन्दी → scroll to the sources. Come back to the PDF for the tables.

3:20 · COMPARISON TABLE + KNOWLEDGE-BASE TABLE [3-min: skip]

Point at the yellow TakeSure column, then the three sample rows on the right.

"Why has nobody built this? Look at the table. Medscape and 1mg do not check herb–drug pairs at all. MSKCC covers single herbs, in English, with no Indian brands. None of them can file a report to the Ministry. TakeSure is the only column with a tick in every row. And on the right is what a knowledge-base row actually looks like: Ashwagandha with levothyroxine — AMBER, grade B, source Sharma 2018. Guggul with warfarin — RED, grade B. Giloy with tacrolimus — RED, grade C. We have 49 such rows with 78 sources today, and every one is auditable."

4:00 · SECTION 4 — EXPECTED IMPACT

Point at the four rows at the bottom.

"Who wins? **Patients and caregivers** get a sourced answer before the first dose, in their own language, free. **Doctors, vaidyas and chemists** finally learn what their patients actually take — a QR code on the chemist's bill or an API inside a hospital system makes the check routine. **The Ministry of Ayush** gets what its pharmacovigilance programme lacks today: structured reports and real data on what India is actually mixing. And the **novelty** is the bridge table itself — Indian brand names, Ayurvedic herb synonyms and graded evidence in one open, auditable table that nobody has built."

4:40 · CLOSE

Look at the camera, not the screen.

"We are not building another health chatbot. We are building the missing safety layer between two medical systems that 1.4 billion people already mix. TakeSure. Lene se pehle, sure ho jao. Thank you — we would love your questions."

For mentoring, replace the last line with the ask: "We would value your guidance on two things: which Ayurveda college or pharmacology department could review our knowledge base, and whether an ABDM / ABHA integration is realistic for the offline round."

Three-minute version (mentoring slot)

- Header (0:15) → Story strip, slightly faster (0:35) → Problem with only two numbers: 19 % and 1,473 (0:30) → Solution, five verbs plus the rule (0:35) → Flowchart *or* live demo, not both (0:50) → Impact in one sentence each (0:20) → Close with the ask (0:15).
- Skip the comparison and knowledge-base tables; if asked "why is this different", answer from the table.

If a judge interrupts with a question

Question	Answer in one breath
Does the AI decide the risk?	No. A rule engine reads cited knowledge-base rows. The AI only rewrites the matched row; with the AI off the verdict is identical.
Where is the data from?	Hand-curated from PubMed, NIH LiverTox, MSKCC About Herbs, WHO monographs and the Ayurvedic Pharmacopoeia; every row has a DOI or PMID and a grade.
What if the herb is not known?	"Not in database." Never green, never a guess. The gap is logged for curation.
Is this medical advice?	No. Decision support: "ask your doctor", never "stop the medicine". Disclaimer on every screen and PDF.

How good is the OCR?	Tesseract English + Hindi, and every name is shown back for confirmation; manual entry always exists.
Privacy?	Photos are deleted after extraction; only an anonymous query, verdict and optional state are stored.
Is it built or an idea?	A working prototype: Next.js front end, FastAPI back end, SQLite knowledge base with 49 interactions and 78 sources, 51 automated tests. We can show it live.
Business model?	Free for citizens; API for pharmacies, hospitals and telemedicine; programmes with the Ministry and CSR from Ayurvedic brands.